

ADVANCED ANALGESIA PROTOCOL																																				
CONFIRM INDICATION																																				
1. Pain score = 2/3 2. Acute and persistent stimuli (E.g. Fractures, dislocations, Burns etc) 3. All other appropriate options have been attempted (See pain management protocol)																																				
<i>If pain score not fully explained by mechanism/injuries, re-examine for missed injuries and seek senior advice.</i>																																				
ESTABLISH REACHBACK																																				
Essential	1. Tempus Pro with Corsium (Y/N) 2. Live voice link to doctor (Y/N)																																			
Desirable	1. Live video link to doctor (Y/N)																																			
CHECK PATIENT																																				
Airway/Breathing	Airway patent (Y/N) O2 Sats..... RR.....																																			
Circulation	Bleeding controlled (Y/N) Palpable radial pulse (Y/N) HR.....(bpm) Cap refill.....(secs) BP.....(mmHg)																																			
Disability	AVPU..... BM.....																																			
Allergies	Y/N What?																																			
Interactions	Confirm all drugs given so far: <table border="1"> <thead> <tr> <th>Drug</th> <th>/</th> <th>Route</th> <th>/</th> <th>Dose</th> <th>/</th> <th>Time</th> </tr> </thead> <tbody> <tr> <td>1.....</td> <td>/</td> <td>.....</td> <td>/</td> <td>.....</td> <td>/</td> <td>.....</td> </tr> <tr> <td>2.....</td> <td>/</td> <td>.....</td> <td>/</td> <td>.....</td> <td>/</td> <td>.....</td> </tr> <tr> <td>3.....</td> <td>/</td> <td>.....</td> <td>/</td> <td>.....</td> <td>/</td> <td>.....</td> </tr> <tr> <td>4.....</td> <td>/</td> <td>.....</td> <td>/</td> <td>.....</td> <td>/</td> <td>.....</td> </tr> </tbody> </table>	Drug	/	Route	/	Dose	/	Time	1.....	/		/		/		2.....	/		/		/		3.....	/		/		/		4.....	/		/		/	
Drug	/	Route	/	Dose	/	Time																														
1.....	/		/		/																															
2.....	/		/		/																															
3.....	/		/		/																															
4.....	/		/		/																															
Access (IV/IO)	Site Secure (Y/N) Flushed (Y/N)																																			
<i>Abnormal findings may impact drug/dose selection. Discuss any concerns with MO</i>																																				
CHECK EQUIPMENT																																				
Essential	1. Airway devices (OPA, NPA, I-gel) (Y/N) 2. BVM (Y/N) 3. Suction (Y/N) 4. Rescue drug available (Y/N)																																			
Desirable	1. Oxygen supply (Y/N) 2. O ₂ saturations (Y/N) 3. ECG (Y/N) 4. BP (cycle 2 minutes) (Y/N) 5. End tidal CO ₂ with waveform (Y/N)																																			

SELECT AND GIVE DRUG (MO to advise)		
MORPHINE 10mg/ml in 1ml Ampoule		
Preparation	Initial dose	Subsequent dose
Dilute 10mg (1ml) in 9ml of NACL to make 10mls at 1mg/ml	5mg (5ml) loading dose THEN 2.5mg (2.5ml) every 15 minutes Max initial dose = 20mg IF FENTANYL HAS BEEN GIVEN NO 5MG LOADING DOSE 2.5mg (2.5ml) every 15 minutes Max initial dose = 10mg	Up to 20mg every 8 hrs Maximum of 60mg in 24hrs
KETAMINE 10mg/ml in 20ml Vial		
Preparation	Initial dose	Subsequent dose
Draw up initial expected dose in 5ml syringe (undiluted)	Start at 10-20mg (1–2ml) slow IV push. Repeat with 10mg (1ml) every 5 mins Max initial dose = 0.5mg/kg e.g. for 80 kg adult max total dose = 40mg	Not routinely advised. Risk of accidental sedation. Discuss maintenance plan with MO
DOCUMENT		
1. Drug, dose, time and route. 2. Pre and post pain score 3. Observations 4. Analgesic plan (What, when, regular and breakthrough)		
MONITOR FOR COMPLICATIONS		
MORPHINE		
Respiratory depression	1. Stop administration. 2. Give O2. 3. Use airway adjunct and jaw thrust +/- BVM (if apnoeic) until opiates have worn off. 4. If above not effective or patient continuing to deteriorate consider Naloxone (see Naloxone box).	
Hypotension	1. Stop administration. 2. Raise legs if safe to do so. 3. Consider fluid bolus. 4. Consider Naloxone (see Naloxone box). 5. If rapid and severe, consider anaphylaxis.	
Drowsiness/sedation	1. Stop administration. 2. Consider Naloxone (see Naloxone box)	
Nausea and vomiting	1. Give Ondansetron 4mg IV 2. Suction +/- Postural drainage (if safe to do so)	

Itch	1. In isolated itch +/- rash, consider Anti-histamines. 2. If signs of true allergic reaction, also stop administration. 3. If developing severe response (with cardiorespiratory Sx) follow anaphylaxis protocol.	
KETAMINE		
Laryngospasm	1. Stop any painful stimuli. 2. 100% O2 + Jaw thrust + double grip vent with BVM 3. If above fail and complete airway obstruction present (hypoxia, absent EtCO2, bradycardia) consider repeat ketamine dose to achieve emergency sedation = <u>rescue technique only</u>	
Apnoea	1. Stop administration. 2. 100% O2 plus BVM (2 person) 3. Caution re. using adjuncts or instrumenting airway due to risk of laryngospasm.	
Excessive secretions	1. Suction +/- Postural drainage (if safe to do so)	
Emergence phenomena	1. Reassure patient and reduce environmental stress 2. Stop painful stimuli 3. Consider Midazolam if severe (see Midazolam box)	
Hypertension/tachycardia	1. Stop any painful stimuli. 2. Consider Midazolam if severe (see Midazolam box)	
Vomiting	1. Give Ondansetron 4mg IV 2. Suction +/- Postural drainage (if safe to do so)	
RESCUE DRUGS		
NALOXONE 400mcg/ml in 1ml Ampoule		
Preparation	Initial dose	Subsequent dose
For IV/IO Dilute 800mcg (2ml) in 6ml NaCl to make 100mcg/ml	For IV/IO 400mcg (4ml)	For IV/IO 100mcg (1ml) every 1 min .
For IM Give undiluted	For IM 400mcg (1ml)	For IM 100mcg (0.25ml) every 2-3 mins IM
Titrate carefully to effect. Aim to reverse significant respiratory depression/drowsiness without reversing analgesic effect.		
MIDAZOLAM 1mg/ml in 5ml Ampoule		
Preparation	Initial dose	Subsequent dose
Draw up initial expected dose in 5ml syringe (undiluted)	0.5mg (0.5ml) IV/IO	Repeat dose every 5-10 mins up to 2mg (2ml) <u>only if required</u>
Use smallest effective dose. Use extreme caution if opiates on board.		